



# Intake Meeting, Case Allocation and Case Review (Mental Health) Policy

## Preamble

### Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to<sup>1</sup>:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

### Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

## Intake meeting

- All new community referrals will be discussed by the multidisciplinary team (MDT) at the Intake meeting on the next working day, to determine the appropriate service for the client. Triage of referrals will be prioritised according to acuity and risk. Staff are to refer to:
  - \* [South Metropolitan Health Service \(SMHS\) Triage assessment of referrals in community mental health](#)
  - \* RKPG Mental Health Service (MHS) Assessment and Treatment Team (ATT) Functional Brief for information relating to the structure and function of the service.
  - \* The intake meeting will function as an MDT, and, as a minimum, be comprised of one medical staff member, one ATT staff member and one Continuing Treatment Team (CTT) staff member.
- All new referrals are to be entered into the triage function on the psychiatric services online information system (PSOLIS) prior to the Intake meeting.
- At the Intake Meeting:
  - \* The triage officer or delegate will present new referrals to the MDT and discuss the initial triage assessment, based on the client's need, appropriateness for MHS and risk/urgency (full initial assessment may have already been carried out by another clinician within the service e.g. Emergency Department (ED) psychiatric liaison nurse (PLN)).
  - \* The MDT will determine the outcome of the referral. If not accepted, the MDT may make recommendations of naming more suitable services to fit the client's needs.
  - \* Clinical responsibility for further assessment will be allocated by the MDT.

| RESPONSIBILITY    |                                                                            |              |                    | NATIONAL STANDARDS                                    |
|-------------------|----------------------------------------------------------------------------|--------------|--------------------|-------------------------------------------------------|
| First Issued      | Jan 2007                                                                   | Revisions    | May 2018, Dec 2020 | Mental Health National Standards<br>1, 3, 6, 8, 9, 10 |
| This version      | Oct 2022                                                                   | Revision Due | Oct 2025           |                                                       |
| Responsibility of | Co-Directors<br>Mental Health                                              | Endorsed by  | DONM, DCS          |                                                       |
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- \* Clinical responsibility for interim care coordination will be allocated by the MDT, if required.
- \* Referrals which have not been formally activated will be reviewed with the MDT within 3 weeks.
- \* If no further follow-up (NFA) is required, the referral will be closed, and referrer notified of the outcome and rationale for no further follow-up by phone or in writing.
- \* Actions:
  - Rationale for NFA will be documented on the Triage/Intake/Referral Outcome Information Form (RG174) by ticking all relevant boxes/reasons.
  - Rationale for no further follow-up (NFA) and summary of discussion will be recorded in the PSOLIS referral comments section and in the integrated progress notes.
  - PSOLIS administration staff will be provided the Triage/Intake/Referral Outcome Information Form (RG174) at the end of the meeting and enter **all** marked reasons into the relevant PSOLIS service event comments box.

### Urgent referrals

All clients either referred urgently or self-presenting to RKPG MHS will have a triage assessment carried out by the triage officer or another clinician to determine:

- Current needs.
- Level of risk and response required.
- Need for onward referral to another service/agency.
- Cases of a complex nature may require discussion with a senior clinician or medical officer (MO). Availability of a crisis action plan should be addressed via review of PSOLIS information.
- Once triaged, all urgent referrals are then discussed with the MDT as per intake process.

## **Case allocation**

### Community

- The needs of the client are to be reviewed at the intake meeting by the MDT prior to or after comprehensive assessment and the most appropriate clinician allocated based on this review.
- Where referral to a PaRK MHS community team (Intensive Day Therapy Unit (IDTU), Early episode Psychosis (EEP), Assertive Community Treatment (ACT) Team, Specialist Rehabilitation Team (SRT), Kwelena Step Up Step Down (SUSD), Continuing Treatment Team (CTT) is indicated, the referral will be discussed with/referred to that team, based upon the intake process for that team. An interim ATT case manager will be assigned.
- The most appropriate and available clinician in the accepting community team will be allocated as case manager.
- All referrals must be activated on PSOLIS at the earliest opportunity following either initial assessment or medical review if ongoing interventions are required. It is essential

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that all national outcome casemix collection (NOCC) measures required are also entered.

- Actions and summary of discussion will be recorded in the integrated case notes.

### Inpatient

- If a client is admitted to the Rockingham General Hospital (RGH) mental health inpatient unit (MHIU), Mimidi Park, a primary nurse and associate nursing staff will be allocated to support the client during admission, coordinate the client care plan and be the point of contact regarding day to day care and communication, including communication with carer and community case manager (CM), as appropriate.
- For active CCT or other community team clients, the client will remain active during the inpatient admission and the case manager or proxy, will maintain the CM role throughout the inpatient stay and may provide clinical support in-reach during the admission to assist discharge planning.
- Clients within ATT are discharged at the point of admission to hospital and are re-referred upon discharge as deemed clinically appropriate by the inpatient treating team. De-activation and reactivation within PSOLIS will be completed and appropriate health of national outcomes scales (HoNOS) measures entered.
- Actions and summary of discussion will be recorded in the integrated case notes.

## **Case review process**

### Community

- PaRK MHS clients will have their progress reviewed at regular intervals by the MDT. Whilst a community client; the review must occur every three months, as a minimum, with additional clinical reviews to occur in response to clinical deterioration or significant changes in the client's assessed risk.
- All PaRK MHS community MDTs will have a chair (team coordinator/leader or delegate) and a separate person to scribe live into PSOLIS using the Clinical Review function.
- A case summary inclusive of client input (and carer input where possible) will be presented to the MDT for discussion around management over the next 3 months. After consideration of the case summary, the MDT will discuss any additional outstanding issues, barriers to discharge and options for further treatment and management. Any changes to treatment or management of the client, following from the review should be included in the updated plan.
- All case reviews are to be documented live in PSOLIS using the Clinical Review function utilising the headings:
  - \* Background (brief only).
  - \* Risk.
  - \* Diagnosis.
  - \* Mental Health Act (MHA) status.
  - \* Medication.
  - \* Summary (brief over previous 3 months).
  - \* Plan - MDT's recommendation regarding the presented Management Plan.
- The MDT checklist (RG180.1) forms part of the review with the completion of the elements of this discussed as part of the client review.

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- The PSOLIS Clinical Review is to be printed immediately following the MDT review, signed by the case manager and filed in the integrated progress note section of the medical record. There is no need to duplicate an entry in the medical record.

### Inpatient

- Each client within Mimidi Park will be reviewed at the MDT weekly ward round.
- The structure and format of the weekly ward round will be the same as that of the community MDT utilising the clinical review function however the MDT will be reviewing care over the past week and the MDT checklist (RG180.1) will not form part of the inpatient review.

## **PSOLIS Management Plan**

- Treatment support and discharge planning provides clients and their carers and/or support persons a collaborative plan guiding care within the service and forming an agreed upon treatment plan.
- The management plan is to include:
  - \* Brief summary of clients presenting issues on referral and mental health issues over the past 3 months.
  - \* Identifying who the case manager is and their role/actions
  - \* Details regarding medical care being provided.
  - \* Treatment/actions to address clinically significant issues identified by the client and treating team.
  - \* Treatments are to be recovery orientated, evidence based, and strengths based.
  - \* Treatment/actions to address risk identified in the RAMP.
  - \* With consent, involve the client's family/carer/nominated support person.
  - \* Invite the client to sign the management plan.
  - \* Case manager to sign the management plan.
  - \* Copy of the management plan to be offered to the client and document if accepted/not accepted.
  - \* If a client chooses not to be involved in developing their management plan include this decision within the management plan.

### Discharge and transfer

Planning for discharge should ideally occur with a client when developing the initial management plan. When a discharge plan is actualised:

- The client and, where appropriate, carer should be involved in the planning and finalisation of discharge.
- A discharge diagnosis with any ongoing care requirements and a follow up plan should be articulated.
- Early warning signs of relapse and actions to take to for re-referral as required are discussed. Emergency contact numbers should be provided.
- A care transfer summary is to be completed.
- A deactivation NOCC to be completed.
- The Management Plan to be closed.
- Actions and summary of discussion will be recorded in the integrated case notes.

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## Compliance monitoring

Compliance with this guideline will be monitored by clinical incidents via Datix.

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## References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

## Links

[SMHS Triage assessment of referrals in community mental health](#)

[National Standards for Mental Health Services 2010](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

## National Standards for Mental Health Services

|                                     |   |                                     |   |   |                                     |   |                                     |                                     |                                     |
|-------------------------------------|---|-------------------------------------|---|---|-------------------------------------|---|-------------------------------------|-------------------------------------|-------------------------------------|
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